



Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°090/MVEBDB/12/08/2026

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**HIGHLIGHTS** (last 24 hours)

In the last 24 hours, 100 new confirmed cases and 53 deaths (28 community deaths and 25 intra-Ebola Virus Disease (EVD) cases) have been reported in the provinces of Ituri (67 cases), North Kivu (25 cases), Haut Uélé (6), Tshopo (1) and Bas Uélé (1). A new province has been assigned (Bas Uélé: ZS Buta).

ACCUMULATIVE CASE 4,665	ACCUMULATIVE CONFIRMED DEATHS 2,184	LETHALITY 46.8%	PATIENTS IN ISOLATION / CTE 634	ACCUMULATIVE HEALED 965	TRACKING RATES CONTACTS (Of the Day) 84.2%
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Affected Provinces and Affected Health Zones

**6 Provinces Affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé

**54 Health Zones**

Affected Out of 151
(35.7%)

**Summary of key points from the last 24 hours**

In the last 24 hours, 100 new confirmed cases and 53 deaths (28 Community deaths and 25 intra-ETC deaths from Ebola Virus Disease (EVD) have been reported in the provinces of [province name missing] (**67 cases**), **North Kivu (25 cases)**, **Haut-Uélé (6)**, **Tshopo (1)**, and **Bas-Uélé (1)**. Ituri accounts for approximately two-thirds of the [cases missing] new cases.

A new province has been affected, namely Bas Uélé, with one confirmed case reported in the Buta health zone. The case is a male in his twenties (estimated date of death: August 1, 2026).

To date, the Democratic Republic of Congo has recorded 4,665 confirmed cases, including 2,184 deaths, representing a case fatality rate of 46.8%. Ituri remains the epicenter of the epidemic, accounting for 85.7% of cumulative confirmed cases and 67.0% of new cases reported in the last 24 hours.

Forty-seven (47) patients were declared recovered after receiving two negative results for Ebola Virus Disease (EVD) (42 in Ituri and 5 in North Kivu). However, 25 deaths within Ebola Treatment Centers (ETCs) were recorded in Ituri (15) and North Kivu (10).

Thus, since the beginning of the epidemic in the DRC, 54 of the 151 health zones in the six provinces are affected. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23), South Kivu (1/34) and Bas Uélé (1/11).

Distribution of confirmed cases and deaths by affected province

(As of August 12, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	3,979	1,726	43.4%	28/36 (77.8%)	67
North Kivu	553	395	71.4%	12/34 (35.3%)	25
Upper Uélé*	119	55	46.2%	6/13 (46.1%)	6
Tshopo	10	6	60.0%	6/23 (26.0%)	1
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	1	1	100.0%	1/11 (9.1%)	1
Total 4,665 2,184	<i>*The cumulative data has been</i>		46.8%	54/151 (35.7%)	100

harmonized with the management team of the Haut Uélé province.

Confirmed cases and deaths by province and health zone (situation as of August 12, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3979	1726	43.4%	67	9	15	24
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	7	2	28.6%				0
Aungba	10	5	50.0%				0
Bamboo	93	22	23.7%				0
Boga	1	1	100.0%				0
Bunia	1095	333	30.4%	23	4		4
Damascus	21	9	42.9%	1	0		0
Drodro	13	6	46.2%	1	0		0
Fataki	59	30	50.8%	4	1		1
Gethy	2	1	50.0%				0
Kambala	2	0	0.0%				0
Kilo	30	12	40.0%				0
Komanda	42	29	69.0%				0
Lita	169	97	57.4%				0
Logo	11	5	45.5%				0
Lolwa	14	4	28.6%	3	0		0
Mahagi	4	1	25.0%				0
Mambasa	13	6	46.2%				0
Mandima	19	13	68.4%				0

Mangala	122	76	62.3%	2	0		0
Mongbwalu	581	282	48.5%	2	1		1
Nia-Nia	151	88	58.3%	5	1		1
Nizi	509	231	45.4%	7	0		0
Nyankunde	118	35	29.7%	1	0		0
Rimba	9	4	44.4%				0
Rwampara	818	319	39.0%	18	2		2
Tchomia	48	24	50.0%				0
To be ventilated	N / A	84	N / A	N / A	N / A	15	15
North Kivu	553	395	71.4%	25	16	10	26
Blessed	85	65	76.5%	5	3		3
Butembo	107	91	85.0%	2	2	3	5
Goma	1	0	0.0%				0
Kalunguta	10	4	40.0%				0
Katwa	262	180	68.7%	13	9	5	14
Kyondo	14	12	85.7%	1	1	1	2
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%	1	1		1
Masereka	7	4	57.1%				0
Musienene	55	32	58.2%	2	0	1	1
Oicha	5	4	80.0%				0
Vuhovi	4	1	25.0%	1	0		0
Haut-Uélé	119	55	46.2%	6	1	0	1
Boma Mangbetu	22	5	22.7%	2			0
Gombari	1	1	100.0%				0
Isiro	37	19	51.4%	4	1		1
Pawa	17	14	82.4%				0
Rungu	1	0	0.0%				0
Wamba	41	16	39.0%				0
Tshopo	10	6	60.0%	1	1	0	1
Bafwasende	1	0	0.0%				0
Kabondo	3	2	66.7%	1	1		1
Lubunga	1	0	0.0%				0
Makiso-Kisangani	3	3	100.0%				0
Mangobo	1	1	100.0%				0
Wanie-Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	1	1	100.0%	1	1	0	1
Buta	1	1	100.0%	1	1		1
Total	4665	2184	46.8%	100	28	25	53

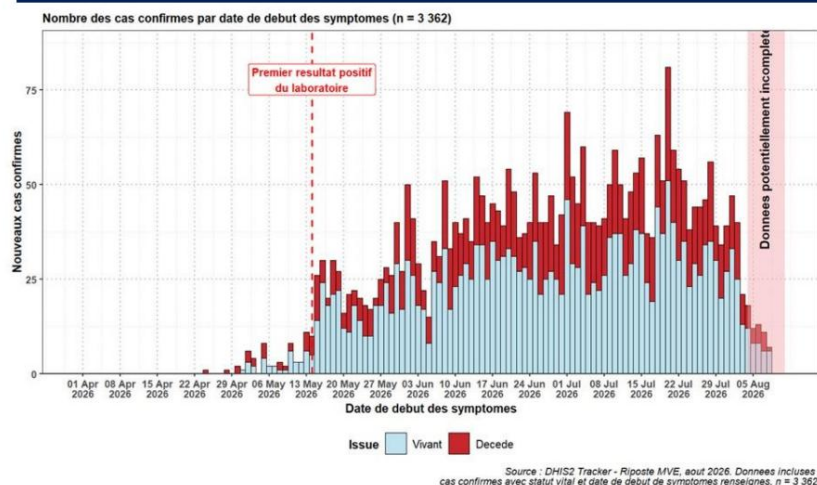
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable

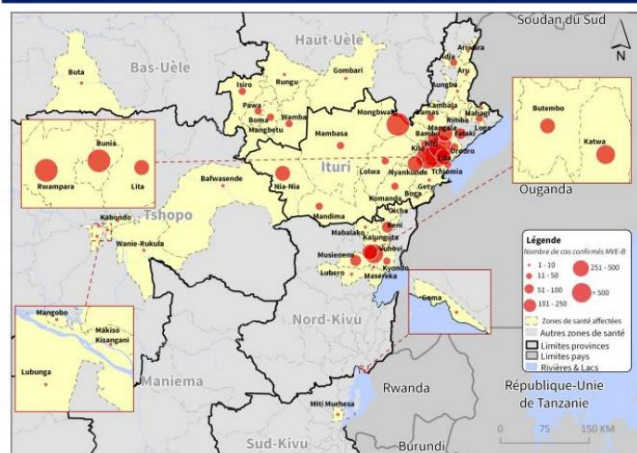
Status of alerts notified by province, as of August 12, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	22	4	26	17	4	5	0	21	14
Ituri	529	39	568	124	38	270	0	122	98
North Kivu	824	58	882	76	58	741	0	132	67
South Kivu	15	0	15	3	0	12	0	3	3
Tshopo	39	1	40	7	1	32	0	8	5
Grand Total 1429		102	1,531	227	101	1060	0	286	187

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Support for the Beni zonal coordination and continuation of the mission support for the Butembo health zone.
- **Haut-Uélé** : arrival in Isiro of the national Incident Manager accompanied by a delegation from the national coordination; holding of two coordination meetings and visit to the isolation center of the HGR of Isiro.

1.2. Epidemiological surveillance

1.2.1. Main Actions

- By the end of the day on August 12, 2026, 1,531 alerts had been recorded, including 1,388 (90.7%) were verified in the 6 reporting provinces. Thus, 328 (23.6%) were validated as suspected cases and 286 (87.2%) were investigated, of which 187 suspects were transferred to CT/CTE.

- The proportion of contacts followed up in the last 24 hours is at 84.2% (17,460 seen out of 20,740 to follow up), close to the threshold of 85%, with lower performance in Haut Uélé (69.7%).

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Two (2) alerts were detected at the different PoE/PoC points (1 in Ituri and 1 in Haut-Uélé): the active alert in Ituri was notified and validated at the Mud'zibala PoC (Nzere Health Zone, Bunia Health Zone) and then referred to the CTE (100%); in Haut-Uélé, an active alert was notified at the Magambe PoC (Isiro Health Zone). No alerts were notified in Tshopo, and North Kivu did not report its PoE/PoC data.
- In total, 158,613 travelers were screened (134,082 in Ituri, 18,379 in Haut-Uélé, and 6,152 in Tshopo), with a screening rate of 97.4%. In Ituri, only one of the 11 strategic Points of Entry/Conversion (PoE/PoC) (9.1%) operated without interruption, and the completeness rate reached 58/60 (96.7%); in Tshopo, 7 out of 20 PoCs were activated (35%), with a completeness rate of 57.1% and 99.3% of travelers screened. North Kivu did not transmit its screening data to the points of entry.
- In Haut-Uélé, 10 PoE/PoC sites are activated out of the 38 priority sites (26.3%), 7 out of 10 sites have transmitted their data (70%), and 18,293 out of 18,379 travelers have been screened (99.5%); the Gossamu PoC alone accounts for 4,105 travelers (22%). The Salisa PoC, between Wamba and Nia-Nia, was activated after the training of service providers.

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Tshopo, with only 7 out of 20 (35%) activated, and reporting completeness is limited to 51 %; in Haut-Uélé, 3 out of 10 PoCs (Matari, Aungba and Dusu) did not report due to lack of network.
- Constraints affecting the functionality of PoE/PoC: 86 refusals to screen (0.5%) were recorded in Haut-Uélé and 38 (0.7%) in Tshopo, motivated by denial of the disease, fear of contamination by handwashing water and fear of being interned at the CTE; in addition to this, there is the non-payment of service providers, the shortage of inputs and personal protective equipment on all activated PoE/PoCs in Haut-Uélé and the lack of means of transport for supervision in Tshopo and Haut-Uélé.

1.4. Laboratory

1.4.1. Main actions

- Ituri recorded 80 positive results (249 samples analyzed), including 67 new cases (58 living and 9 dead) and 13 samples taken, while North Kivu confirmed 25 cases including 16 deaths, Haut-Uélé 6 confirmed cases including 1 death, Tshopo 1 positive case at the Makiso-Kisangani military hospital, and Bas-Uélé 1 case as well. In total, 100 new cases of Ebola virus disease were confirmed in the six provinces; South Kivu recorded no positive results from the 4 samples received.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 19 rings were opened out of 143 (28 reported from the previous day and 115 cases of the day); 70 sites identified and 70 were decontaminated (100%), including 63 households, 6 healthcare facilities and 1 other structure; 43 EDS were carried out out of 59 death alerts in households, 7 out of 8 in healthcare facilities and 20 out of 21 in CTEs, i.e. 70 EDS out of 88 alerts (79.5%), 50 bodies collected post-mortem.
- **In North Kivu**, 49 death alerts were recorded, 49 bodies were recovered and secured without incident, and 44 EDS (Emergency Diagnosis System) tests were carried out, with 5 bodies being deferred until the following day; the deaths were distributed as follows: 37 in healthcare facilities (75.5%), 8 in the community (16.3%), and 4 in Ebola Treatment Centers (8.1%). Two households of confirmed cases were decontaminated in Beni, as well as 13 healthcare facilities where cases were staying (7 in Beni, 3 in Butembo, 2 in Katwa, and 1 in Musienene).
- **In Tshopo**, IPC activities were carried out in 4 health zones (6 rings opened out of 10 planned and 6 care facilities and 2 households decontaminated), with the decontamination of the military pavilion of the Makiso-Kisangani General Referral Hospital and 10 premises of the University Clinics, as well as the dispatching of triage inputs to the care facilities of the Tshopo and Kabondo health zones.
- **In Haut-Uélé**, three death alerts were recorded (one in Wamba, one in Boma Mangbetu, and one in Rungu) with two blood tests (66.7%) and three bodies collected by swab (100%). Decontamination and blood tests resumed in the Wamba health zone after the suspension of Red Cross volunteers was lifted. Decontamination was carried out at the Pinpester and Libiki clinics in Isiro, and infection prevention and control (IPC) kits were provided to Wamba.

1.5.2. Challenges

- Weak implementation of IPC standards: partial decontamination of households of cases in Ituri (63 households decontaminated for 115 confirmed cases of the day), limited completeness of reporting in North Kivu (6 health zones out of 12) as in Ituri (9 health zones out of 12), and low card scores (36.5% on average in North Kivu, 45.2% at the Rekapi Polyclinic and 12.9% at the Makiso-Kisangani military pavilion in Tshopo), demonstrating a continued risk of healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 30 ambulances are available out of 33 allocated to the response: 22 trips successfully completed (100%). The Ebola Treatment Centers (ETCs) recorded 83 admissions and 42 recoveries in the last 24 hours; occupancy reaches 438 patients for 973 beds (45%), i.e. 179 confirmed out of 496 beds (36.1%) and 259 suspected out of 477 beds (54.3%), with saturation of the ETCs in Bambu and Nizi for confirmed cases and in Nyankunde, Fataki, Rwankole and Kigonze for suspected cases.
- **In North Kivu**, there have been 231 cumulative admissions to Ebola Treatment Centers (ETCs); 63 admissions today (6 confirmed and 57 suspected), 5 recoveries, 10 deaths, and 36 suspected but not confirmed cases. Currently, 180 patients are hospitalized (24 confirmed and 146 suspected).
- **In Haut-Uélé**, 11 new admissions (1 confirmed and 10 suspected) were recorded in Boma Mangbetu and 4 recovered patients were discharged to the CTH in Isiro; 53 patients are hospitalized (31 confirmed and 22 suspected) in the ESS for 78 beds set up (67.9%), with exceeding the capacity in Boma Mangbetu and Isiro.
- **In Tshopo**, 2 new admissions (1 confirmed and 1 suspected) bring the number of patients in isolation to 5 out of 12 beds (41.7%), with 17 people having benefited from the experimental treatment in total.

- **In South Kivu**, 11 suspected cases are being monitored in hospital (7 in Katana and 4 in Miti-Murhesa).

1.6.2. Challenges

Insufficient availability of referral resources (3 ambulances out of service in Ituri, no ambulances in 7 health zones of North Kivu, and no TCCs/ETCs in 4 health zones, requiring 10 additional ambulances in Haut-Uélé) and saturation of several TCCs/ETCs, limiting rapid access to care and isolation of suspected cases. In Haut-Uélé, the lack of a standardized TCC persists in the 6 affected health zones, and the capacity of the Wamba temporary isolation center is exceeded; in Ituri, the Kasenyi TCC did not report any cases due to a strike.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, supporting the surveillance pillar, 280 contacts were identified around 16 cases in ten health areas; 1,269 people were reached during home visits, 78 during educational talks, 204 during community dialogues, and 4,774 during mass awareness campaigns
- **In North Kivu**, 200 leaders of motorcycle taxi associations in Butembo were briefed on CREC and PCI; communication actions were carried out around 23 confirmed cases, affecting 202 household leaders, and 14 rumors were clarified for 1 community incident resolved out of 13 recorded.
- **In Tshopo**, 1,051 people were reached during home visits in the Makiso-Kisangani and Tshopo health zones, 72 members of the management teams from 14 rural health zones were trained on the response, and 3 radio programs and 12 public service announcements were broadcast. In Haut-Uélé, 656 people were reached during home visits (220 households visited), 419 during educational talks, and 59 during mass awareness campaigns, on the sidelines of the discharge ceremony for 4 recovered patients at the Isiro General Referral Hospital.

1.7.2. Challenges

Persistence of community challenges: refusal of psychosocial support actions and departure to an unknown destination of a family linked to a community death in the AS of Mungbwalu (Ituri), barriers put up by young people to EDS and decontamination activities in the AS of Pilipili (ZS of Mongbwalu), 4 resistances to EDS in the AS of Mangiva (ZS Nia-Nia) and refusal of sampling in Tshopo (Rekapi and PK 6 airport road), likely to affect community support for the response.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed capacity reached 973 beds (496 for confirmed cases, 477 for suspected cases); delivery of 22 packages of PCI-EDS kits (97 kg) to the Jina health center, PCI kits to the general coordination and surveillance kits to the WHO and the CDC, and supply of 670 liters of fuel; 9 ambulances were deployed in the field; the expansion of the CTEs in Nizi, ISTM and Bambu is requested in the face of saturation.

In North Kivu : Delivery of IPC and epidemiological surveillance supplies to the WHO office in Goma and the Beni health zone; packaging of IPC supplies for the Kalunguta health zone; continued construction of the triage unit at Vuhovi General Referral Hospital (ULB Cooperation); and deployment of three vehicles to support missions. In Tshopo, receipt of 49 kg of chlorine (MSF) and monitoring of the isolation projects in Kabondo (deadline 15/08) and Madula.

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security

1.9.1. Main actions

- Securing the response activities continued, including support for EDS and decontamination teams and the deployment of 3 vehicles to support field missions; the safety and security sections of Ituri and North Kivu did not transmit any figures for the day of August 12, 2026.

1.9.2. Challenges

- Persistent insecurity and community resistance during dignified burials and secured, limiting access for response teams: attacks on teams in burial areas in North Kivu, opposition from young people to response activities in the Pilipili AS (Mongbwalu ZS, Ituri) and failure to take charge of police elements assigned to the CTEs of Katwa and Kitumba.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

• Ebola virus transmission remains active, with 100 new cases and 53 deaths, including 28 fatalities

Community and 25 intra-CTE cases were recorded in the last 24 hours, bringing the national total to 4,665 confirmed cases and 2,184 deaths. • A new province affected: Bas Uélé (Buta), bringing the number of affected provinces to 6, with 54 health zones.

• Ituri remains the epicenter of the epidemic, concentrating 85.7% of cumulative confirmed cases and 67.0% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).

• Surveillance has performed well over the past 24 hours, with 87.2% of suspected cases investigated, a follow-up rate of 84.2% close to the threshold of 85.0%.

• Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

• Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



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